PRELIMINARY EXPERT RESEARCH MEMORANDUM / REVIEW COPY

Boehringer Ingelheim Pharmaceuticals Inc. v. FDC Limited

The public complaint analysis reports that FDC submitted ANDA No. 220862 seeking approval for 10 mg and 25 mg empagliflozin tablets before expiration of four asserted Jardiance method-of-treatment patents. The likely technical center is what FDC's proposed label directs or encourages clinicians to do: assess renal function, treat patients with type 2 diabetes in the claimed estimated-glomerular-filtration-rate range, discontinue or modify treatment at lower renal function, and treat adolescents aged 10 to 17. Expert work will likely require close comparison of the claims, proposed label, renal-function methods, clinical-practice context, and prior art for the claimed patient subgroups and treatment steps.

Independent preliminary research based on public information.

FILED	FORUM	DOCKET	MATTER
2026-08-21	U.S. District Court for the District of New Jersey	3:26-cv-10744	Patent - Abbreviated New Drug Application (Anda)

Prepared 2026-08-25. No attorney or expert contacted. Availability, interest, compensation, and conflicts not checked.

01 / MATTER VERIFICATION AND ASSESSMENT

Public filing, likely recipient, and technical frame

CAPTION	Boehringer Ingelheim Pharmaceuticals Inc. et al. v. FDC Limited et al.
FORUM / DOCKET	U.S. District Court for the District of New Jersey / 3:26-cv-10744
FILED / TYPE	2026-08-21 / Patent - Abbreviated New Drug Application (ANDA)
PUBLIC DOCKET	Open docket

VERIFIED PUBLIC RECORD

The public docket identifies Boehringer Ingelheim Pharmaceuticals Inc., Boehringer Ingelheim International GmbH, and Boehringer Ingelheim Pharma GmbH & Co. KG as plaintiffs and FDC Limited and FDC Inc. as defendants. It records an August 21, 2026 complaint, civil cover sheet, report on the filing or determination of an action regarding a patent or trademark, and five exhibits. The docket identifies Liza M. Walsh of Walsh Pizzi O'Reilly Falanga LLP as counsel for all three plaintiffs.

LIKELY RESEARCH PURCHASER

Liza M. Walsh - Outside counsel, Walsh Pizzi O'Reilly Falanga LLP. Analytical inference: the exact public docket names Walsh as counsel for all three Boehringer plaintiffs, and her public professional biography describes complex commercial and intellectual-property litigation. This does not establish internal staffing or purchasing responsibility.

lwalsh@walsh.law - publicly printed in the linked professional source; not inferred.

TECHNICAL FRAME

The public complaint analysis reports that FDC submitted ANDA No. 220862 seeking approval for 10 mg and 25 mg empagliflozin tablets before expiration of four asserted Jardiance method-of-treatment patents. The likely technical center is what FDC's proposed label directs or encourages clinicians to do: assess renal function, treat patients with type 2 diabetes in the claimed estimated-glomerular-filtration-rate range, discontinue or modify treatment at lower renal function, and treat adolescents aged 10 to 17. Expert work will likely require close comparison of the claims, proposed label, renal-function methods, clinical-practice context, and prior art for the claimed patient subgroups and treatment steps.

Secondary-source limitation: The complaint analysis is a secondary public source, not a substitute for the filed complaint and exhibits, FDC's Paragraph IV notice, the final or proposed FDC label, complete patent prosecution histories, expert reports, or discovery. No conclusion is drawn on infringement, inducement, validity, enforceability, claim scope, regulatory outcome, or commercial outcome. Open public synopsis

REPORTED ASSERTED PATENTS

U.S. Patent No. 11,090,323 | U.S. Patent No. 11,833,166 | U.S. Patent No. 12,433,906 | U.S. Patent No. 12,527,810

Questions likely to require expert input

This issue map is a research plan, not a legal conclusion. The filed complaint, patent exhibits, intrinsic record, and discovery must control.

01 Does FDC's proposed label direct or encourage performance of every asserted treatment step?

A clinical pharmacology or diabetes expert can compare the exact prescribing instructions, warnings, renal-function language, dose options, and patient populations with each asserted claim while separating what the label requires from what clinicians may independently choose.

Potential evidence: Proposed and final FDC labels; Jardiance label history; ANDA correspondence; medical-review materials; asserted claims; prescribing guidelines; physician testimony; formularies and clinical pathways

02 How is estimated glomerular filtration rate assessed in ordinary diabetes and kidney care?

A nephrologist can explain eGFR equations, input measurements, reporting conventions, timing, repeat testing, uncertainty near the claimed thresholds, and how renal-function assessment relates to starting, continuing, or stopping empagliflozin.

Potential evidence: Laboratory methods and reports; CKD-EPI or other equations; label instructions; medical records; clinical guidelines; regulatory reviews; protocol and standard-of-care evidence

03 What treatment acts are clinically meaningful for patients with type 2 diabetes and eGFR at least 30 but below 60?

An expert can address glycemic efficacy, kidney and cardiovascular effects, dose selection, monitoring, contraindications, and the clinical meaning of improving glycemic control in the claimed renal-function subgroup.

Potential evidence: Empagliflozin trials and subgroup analyses; FDA reviews; prescribing information; guidelines; real-world studies; renal and glycemic outcome measures; adverse-event data

04 What does the intrinsic and clinical record mean by discontinuation or treatment modification below an eGFR threshold?

The analysis may require distinguishing a claimed treatment sequence from safety monitoring, temporary interruption, permanent discontinuation, changing regulatory recommendations, and clinical decisions driven by acute versus chronic renal changes.

Potential evidence: Claim language and prosecution histories; label versions; regulatory communications; clinical protocols; adverse-event management guidance; expert explanation of renal-function trajectories

Questions likely to require expert input

05 How does the adolescent claim map onto treatment of patients aged 10 through 17?

A pediatric endocrinologist can address diagnosis and treatment of adolescent type 2 diabetes, dosing and monitoring of empagliflozin, developmental and safety considerations, and whether the proposed label identifies the claimed age-defined population and acts.

Potential evidence: Pediatric label sections; DINAMO and related studies; pediatric diabetes guidelines; age-stratified safety and efficacy data; regulatory pediatric assessments; proposed indication language

06 What did the prior art teach about empagliflozin use in the claimed renal-function and adolescent subgroups?

Validity analysis will likely require a fixed-date reconstruction of clinical expectations, label knowledge, renal impairment data, pediatric development, and whether the claimed combinations of patient selection, assessment, treatment, and discontinuation were disclosed or would have been predictable.

Potential evidence: Priority-date patents and publications; historical labels; guidelines; trial protocols and results; regulatory meeting materials; pediatric study plans; contemporaneous clinician surveys and textbooks

Candidates 1-2 for further diligence

Ranked for preliminary research priority based on subject-matter fit, relevant public work, testimony, and visible diligence burden. This is not a retention recommendation.

01 David Z. I. Cherney

Professor of Medicine and Physiology, University of Toronto; nephrologist and Director of the Renal Physiology Laboratory, University Health Network

TECHNICAL FIT

Top fit for empagliflozin and SGLT2 renal physiology, eGFR-related treatment effects, diabetic kidney disease, and translation between clinical-trial evidence and label-directed practice.

RELEVANT WORK

His University of Toronto profile places him in nephrology, physiology, and the Banting & Best Diabetes Centre. His published work addresses SGLT2 inhibitors, renal hemodynamics, glycosuria, and management of diabetic kidney disease.

PUBLIC LITIGATION EXPERIENCE

No candidate-specific expert report, deposition, trial testimony, or admissibility decision was located in the bounded public search.

POINTS FOR FURTHER DILIGENCE

His extensive SGLT2 clinical research may include relationships with manufacturers, including companies associated with empagliflozin; all trial, consulting, funding, and publication ties require focused conflicts and cross-examination diligence. He is not a pediatric specialist or labeling expert.

Source 1 | Source 2

Draft email | david.cherney@uhn.ca | public email source

02 Hiddo J. L. Heerspink

Professor of Clinical Pharmacology, University Medical Center Groningen

TECHNICAL FIT

Excellent clinical-pharmacology and kidney-trial fit for SGLT2 treatment effects, albuminuria and eGFR endpoints, renal-subgroup evidence, dose-response, and the interpretation of label-supporting trials.

RELEVANT WORK

UMCG identifies him as a professor conducting drug studies and personalized medicine in diabetic kidney disease. He has led major kidney-outcomes analyses and SGLT2 trials, including work on dapagliflozin and canagliflozin.

PUBLIC LITIGATION EXPERIENCE

No candidate-specific expert report, deposition, trial testimony, or admissibility decision was located in the bounded public search.

POINTS FOR FURTHER DILIGENCE

His extensive industry-sponsored trial portfolio creates a substantial relationship-diligence burden. His closest published work is often on SGLT2 agents other than empagliflozin, and he is not a pediatric endocrinologist or U.S. regulatory-label specialist.

Source 1 | Source 2 | Source 3

Draft email | h.j.lambers.heerspink@umcg.nl | public email source

Candidates 3-4 for further diligence

03 Christoph Wanner

Senior Professor of Medicine, University Hospital of Würzburg; Visiting Professor, Oxford Population Health

TECHNICAL FIT

Direct empagliflozin fit for diabetic-kidney outcomes and the clinical interpretation of renal-function evidence, including EMPA-REG and later kidney studies.

RELEVANT WORK

Oxford identifies him as a steering-committee member for empagliflozin studies and notes the landmark EMPA-REG outcome and renal analyses, alongside extensive diabetic-kidney-disease research.

PUBLIC LITIGATION EXPERIENCE

No candidate-specific expert report, deposition, trial testimony, or admissibility decision was located in the bounded public search.

POINTS FOR FURTHER DILIGENCE

His direct involvement in empagliflozin studies makes manufacturer, sponsor, publication, and advocacy relationships an especially important conflicts issue. He is based in Europe and may be closer to outcome evidence than U.S. label interpretation.

Source 1

Draft email | christoph.wanner@ndph.ox.ac.uk | public email source

04 Silvio E. Inzucchi

Professor of Medicine and Clinical Chief of Endocrinology, Yale School of Medicine; Medical Director, Yale Diabetes Center

TECHNICAL FIT

Strong diabetes-treatment and empagliflozin cardiovascular-outcomes fit for standard of care, glycemic-control meaning, patient selection, and how clinicians read and act on antihyperglycemic labels.

RELEVANT WORK

Yale reports leadership in large trials of diabetes medications and cardiovascular impact, extensive work on pharmacologic antihyperglycemic therapy, and authorship of major diabetes-management position statements.

PUBLIC LITIGATION EXPERIENCE

No candidate-specific expert report, deposition, trial testimony, or admissibility decision was located in the bounded public search.

POINTS FOR FURTHER DILIGENCE

His trial and advisory relationships with diabetes-drug manufacturers require full diligence, especially any empagliflozin or Boehringer/Lilly connections. He is not primarily a nephrologist or pediatric diabetes specialist.

Source 1

Draft email | silvio.inzucchi@yale.edu | public email source

Candidates 5-6 for further diligence

05 Katherine R. Tuttle

Clinical Professor of Medicine, University of Washington; Executive Director for Research, Providence Health Care

TECHNICAL FIT

High fit for diabetic kidney disease, eGFR thresholds, kidney-protection evidence, SGLT2 guideline context, and the relationship between clinical-trial endpoints and real-world renal care.

RELEVANT WORK

Her University of Washington profile lists diabetic kidney disease and novel therapies as core research interests, service on the EMPA-KIDNEY steering committee, and co-chairing an SGLT2-inhibitor workshop in diabetic kidney disease.

PUBLIC LITIGATION EXPERIENCE

No candidate-specific expert report, deposition, trial testimony, or admissibility decision was located in the bounded public search.

POINTS FOR FURTHER DILIGENCE

Her broad clinical-trial and advisory activity creates sponsor and consulting diligence. Publicly identified involvement in Boehringer-related kidney research may create a direct conflict concern that must be assessed before any outreach.

Source 1 | Source 2

Draft email | katherine.tuttle@providence.org | public email source

06 David C. Wheeler

Professor of Kidney Medicine, University College London; Head of the Department of Renal Medicine, UCL

TECHNICAL FIT

Strong CKD-trial and SGLT2 fit for renal outcomes, eGFR assessment, treatment thresholds, clinical guidelines, and comparison of trial populations with claimed subgroups.

RELEVANT WORK

UCL describes his leadership of randomized trials of new CKD therapies, including SGLT2 inhibitors, and identifies him as academic co-chair of DAPA-CKD with extensive work on kidney and cardiovascular outcomes.

PUBLIC LITIGATION EXPERIENCE

No candidate-specific expert report, deposition, trial testimony, or admissibility decision was located in the bounded public search.

POINTS FOR FURTHER DILIGENCE

His most visible SGLT2 leadership centers on dapagliflozin rather than empagliflozin. Industry trial roles, sponsors, advisory work, and international location require diligence; pediatric and U.S. labeling issues are outside his core focus.

Source 1 | Source 2

Draft email | d.wheeler@ucl.ac.uk | public email source

Candidates 7-8 for further diligence

07 Vlado Perkovic

Provost and Scientia Professor, UNSW Sydney; nephrologist and kidney-trial investigator

TECHNICAL FIT

Strong fit for kidney-outcome trials, progression of CKD, interpretation of renal endpoints, and whether evidence from SGLT2 studies supports treatment expectations in the claimed eGFR subgroup.

RELEVANT WORK

UNSW states that his research focuses on trials and epidemiology to prevent kidney-disease progression and that he has led major international studies and kidney-disease guidelines.

PUBLIC LITIGATION EXPERIENCE

No candidate-specific expert report, deposition, trial testimony, or admissibility decision was located in the bounded public search.

POINTS FOR FURTHER DILIGENCE

He is a senior academic administrator with broad trial relationships and potential scheduling constraints. Much of his SGLT2 work involves canagliflozin or class-wide evidence rather than the specific asserted empagliflozin methods.

Source 1

Draft email | vlado.perkovic@unsw.edu.au | public email source

08 Lori M. Laffel

Professor of Pediatrics, Harvard Medical School; Senior Investigator and Chief of the Pediatric, Adolescent, and Young Adult Section, Joslin Diabetes Center

TECHNICAL FIT

Best pediatric fit for the patent directed to patients aged 10 through 17, adolescent diabetes care, treatment adherence, safety, age-specific clinical practice, and the meaning of a pediatric label indication.

RELEVANT WORK

Joslin identifies her as a pediatric endocrinologist and clinical investigator whose work covers children, adolescents, and young adults, including treatment of type 2 diabetes in youth and prevention of diabetes complications.

PUBLIC LITIGATION EXPERIENCE

No candidate-specific expert report, deposition, trial testimony, or admissibility decision was located in the bounded public search.

POINTS FOR FURTHER DILIGENCE

Her strongest public record concerns pediatric diabetes broadly and often type 1 diabetes, not specifically adolescent empagliflozin or renal-function claims. Industry, trial, and guideline relationships require diligence.

Source 1 | Source 2

Draft email | lori.laffel@joslin.harvard.edu | public email source

Candidates 9-10 for further diligence

09 John P. H. Wilding

Professor of Medicine, Cardiovascular and Metabolic Medicine, University of Liverpool

TECHNICAL FIT

Strong type 2 diabetes pharmacotherapy fit for SGLT2 positioning, glycemic control, patient selection, dose and benefit-risk context, and ordinary clinician response to label language.

RELEVANT WORK

Liverpool reports leadership of clinical research in obesity, diabetes, and endocrinology, more than 350 publications, extensive diabetes-drug trials, and work specifically positioning SGLT2 inhibitors in the treatment algorithm.

PUBLIC LITIGATION EXPERIENCE

No candidate-specific expert report, deposition, trial testimony, or admissibility decision was located in the bounded public search.

POINTS FOR FURTHER DILIGENCE

His expertise is broader diabetes management rather than renal physiology, pediatric care, patent disclosures, or U.S. label regulation. Clinical-trial, consulting, and manufacturer relationships require full diligence.

Source 1 | Source 2

Draft email | J.P.H.Wilding@liverpool.ac.uk | public email source

10 Peter Rossing

Head of Clinical and Translational Research, Steno Diabetes Center Copenhagen; Professor, University of Copenhagen

TECHNICAL FIT

Strong diabetic-kidney-disease and SGLT2 fit for renal and cardiovascular risk, subgroup evidence, clinical guidelines, treatment monitoring, and the significance of eGFR-based care decisions.

RELEVANT WORK

Steno identifies his focus on diabetic kidney and cardiovascular complications, clinical trials of kidney treatments, and steering-group roles for DAPA-CKD and other major cardiorenal trials; his current work includes SGLT2 inhibitors.

PUBLIC LITIGATION EXPERIENCE

No candidate-specific expert report, deposition, trial testimony, or admissibility decision was located in the bounded public search.

POINTS FOR FURTHER DILIGENCE

His extensive work with pharmaceutical-sponsored trials and treatment guidelines creates meaningful conflicts and impeachment diligence. He is not a pediatric specialist or U.S. labeling expert, and his work often addresses class effects rather than a single product.

Source 1 | Source 2

Draft email | peter.rossing@regionh.dk | public email source

04 / DILIGENCE, SERVICE, AND SOURCES

Preliminary candidates; no conflicts or availability determination

- Run party, affiliate, counsel, inventor, funder, and technology conflicts.

- Confirm testimony history, exclusions, compensation, publications, patents, and deposition performance.

- Investigate licensing, grants, consulting, commercial interests, and prior technical positions.

- Confirm role fit, availability, and interest only after counsel authorizes outreach.

ONGOING MATTER-SPECIFIC RESEARCH

Flint Witness Research works in the post-filing window, turning pleadings and public technical records into issue maps, researched candidate slates, and diligence starting points as the case develops.

Schedule a 15-minute conversation

CORE SOURCES

Public docket: https://dockets.justia.com/docket/new-jersey/njdce/3%3A2026cv10744/606474

Public professional email source: https://walsh.law/wp-content/uploads/Walsh-1.pdf

Public technical context source: https://ai-lab.exparte.com/case/dct/njd/3%3A26-cv-10744/doc/analysis/1

LIMITATIONS

The complaint analysis is a secondary public source, not a substitute for the filed complaint and exhibits, FDC's Paragraph IV notice, the final or proposed FDC label, complete patent prosecution histories, expert reports, or discovery. No conclusion is drawn on infringement, inducement, validity, enforceability, claim scope, regulatory outcome, or commercial outcome.

No attorney or expert was contacted. Availability, interest, compensation, conflicts, admissibility, and retention suitability were not checked.